

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 16

Age: 49 years old

Sex: Female

MRN: 000-TEST-16

Admission Date: [Current Date - 4 days]

Discharge Date: [Current Date]

Attending Physician: Dr. M. Richardson, MD

****PRIMARY DIAGNOSIS****

Acute exacerbation of Chronic Obstructive Pulmonary Disease (COPD), GOLD Stage 2 (moderate airflow limitation). FEV1/FVC ratio 0.58 on admission spirometry.

****SECONDARY DIAGNOSES****

- Hypothyroidism, well-controlled on levothyroxine
- Generalized Anxiety Disorder
- Mild oxygen desaturation on exertion (baseline SpO2 92-94%)

****HOSPITAL COURSE****

Patient 16 presented to Metro Medical Center via ED with acute dyspnea on exertion (DOE), productive cough × 3 days, and increased sputum volume. Initial vital signs: HR 102, RR 24, BP 128/82, SpO2 88% on room air. Chest X-ray showed mild hyperinflation without acute infiltrate. ABG revealed mild hypoxemia (pO2 76 mmHg, pCO2 42 mmHg).

Hospital management included:

- Supplemental O2 titrated to target SpO2 $\geq$ 90%
- Nebulized albuterol/ipratropium q.4h $\times$ 48h, then q.6h
- IV methylprednisolone 125 mg q.6h $\times$ 2 days, then transitioned to oral prednisone taper
- Chest physiotherapy and breathing exercises
- Patient education on inhaler technique (observed demonstration performed)

SpO2 improved to 94-96% on 1-2 L NC by hospital day 3. Patient discharged in stable condition with SpO2 95% on room air at rest.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
------------	------	-----------	----------

---	---	---	---
-----	-----	-----	-----

Tiotropium (LAMA)	18 mcg	1 puff daily	Ongoing
-------------------	--------	--------------	---------

Formoterol/Budesonide (LABA/ICS)	120/2 mcg	2 puffs BID	Ongoing
----------------------------------	-----------	-------------	---------

Albuterol MDI	90 mcg	1-2 puffs q.4-6h PRN	PRN
---------------	--------	----------------------	-----

Prednisone (taper)	40 mg $\rightarrow$ 20 mg $\rightarrow$ 10 mg	Daily $\times$ 3 days each	9 days total
--------------------	---	----------------------------	--------------

Levothyroxine	75 mcg	1 tablet q.d.	Ongoing
---------------	--------	---------------	---------

Sertraline	100 mg	1 tablet q.d.	Ongoing
------------	--------	---------------	---------

Ipratropium/Albuterol neb	0.5/2.5 mg	q.6h PRN	PRN
---------------------------	------------	----------	-----

****ACTIVITY & RESTRICTIONS****

- No heavy lifting or strenuous exertion $\times$ 2 weeks
- Light ambulation as tolerated; encourage gradual increase in activity

- May return to normal activities as tolerated once prednisone taper complete

****DIETARY****

No restrictions. Encourage adequate hydration (2 L fluids daily if tolerated).

****FOLLOW-UP APPOINTMENTS****

1. ****Pulmonology****: Dr. K. Patel, MD — in 3-4 weeks (post-exacerbation reassessment, repeat spirometry, GOLD classification confirmation)
2. ****Primary Care****: Dr. R. Martinez, MD —